INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Serplulimab, carboplatin and etoposide

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00908
Version 1b

Treatment stages

01 **Induction · four planned cycles**
Three medicines

02 **Maintenance · after induction**
One medicine

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	6
03	Symptoms and supportive care	12
04	Sources and further information	23

My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. New cough, diarrhoea, severe weakness, marked thirst, reduced urine or blistering skin need prompt assessment. Severe breathing difficulty, chest pain, collapse, seizures or major bleeding need emergency help. Keep the immunotherapy alert card after treatment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

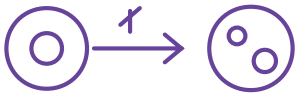
Three medicines. Different jobs.

This treatment aims to control extensive-stage small-cell lung cancer, relieve symptoms and help people live longer. The balance of benefit, side effects and daily life is individual; it is generally not offered with an expectation of cure.

IMMUNOTHERAPY

Serplulimab

Hetronifly



Blocks the PD-1 immune brake

Serplulimab is an antibody that blocks PD-1, a control switch on immune T cells. Some cancers use PD-L1 or PD-L2 to engage this switch and reduce the immune response. Blocking it can help immune cells attack cancer. The same immune activation can inflame healthy organs, so new symptoms need assessment even after treatment stops.

Immune cell → Brake blocked → Cancer cell

What to watch for

The immune system can also inflame healthy organs. New cough, diarrhoea, unusual weakness, severe headache, thirst or a spreading rash need prompt advice. Effects can start during treatment or after it finishes.

CHEMOTHERAPY

Carboplatin

Platinum chemotherapy



Damages cancer-cell deoxyribonucleic acid (the cell's genetic material) (DNA)

Carboplatin is a platinum-containing chemotherapy. Inside cells, it forms chemical attachments to DNA, including links that stop DNA strands working normally. This disrupts copying and repair and can lead to cell death. Healthy cells can also be injured, especially blood-forming cells in the bone marrow. The kidneys help remove carboplatin, so kidney function is used to calculate the dose. AUC 5 describes a target drug exposure over time; it is not a fixed dose of 5 mg.

DNA strands → Platinum links → Copying disrupted

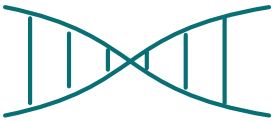
What to watch for

Low blood counts, nausea and vomiting can occur. An allergic reaction is also possible: tell the nurse immediately if you feel flushed, itchy, tight-chested or unwell during the drip.

CHEMOTHERAPY

Etoposide

Chemotherapy; intravenous (IV) or a separately prescribed capsule alternative



Interferes with DNA repair during cell division

Etoposide blocks topoisomerase II, an enzyme that helps untangle DNA. It can leave DNA breaks unrepaired so that dividing cancer cells die. Healthy dividing cells are affected too, which can lower blood counts, irritate the mouth and bowel and cause hair loss. Kidney function, liver tests and albumin help the team choose a safe dose. Capsules have different dosing instructions from the drip.

DNA strands → Repair disrupted → Cell division stops

What to watch for

Report fever or sudden illness immediately. During a drip, tell the nurse about dizziness, flushing, breathing difficulty or pain at the cannula. If capsules are prescribed, do not take an extra dose after vomiting or a missed dose without advice.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the treatment aim

Take one section at a time and discuss what matters to you.

What do the words mean?

Small-cell describes the cancer cells seen under the microscope. Extensive-stage means it is too widespread for the limited-stage treatment approach. First-line means the first drug treatment for this stage. This regimen combines two chemotherapy medicines with serplulimab immunotherapy.

What is treatment trying to achieve?

The usual aims are cancer control, symptom relief and longer life while considering quality of life. It is generally not given with an expectation of cure. Some people have a prolonged response; others do not. Your team uses your cancer tests, general health and preferences to choose a plan.

What are the alternatives?

Ask about other appropriate systemic treatments, a clinical trial and symptom-focused care. Palliative care can support symptoms and daily life while you receive active cancer treatment. This regimen has no required PD-L1 cutoff; that does not guarantee benefit.

Why this treatment is offered

A randomised study found that adding serplulimab to carboplatin and etoposide helped people with previously untreated extensive-stage small-cell lung cancer live longer than chemotherapy alone.

These results describe groups. They cannot predict your own response or length of life. Ask what benefit is realistic for you and what matters most in your daily life.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Induction → review → maintenance

Treatment stages

01 **Induction · four planned cycles**
Three medicines

02 **Maintenance · after induction**
One medicine

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

THREE MEDICINES

Induction · four planned cycles

C1 C2 C3 C4

Induction lasts four planned 21-day cycles unless treatment needs to stop earlier. On Day 1 you receive serplulimab, carboplatin and etoposide by drip. With the standard IV schedule, etoposide is repeated on Days 2 and 3.

ONE MEDICINE

Maintenance · after induction

M1 M2 M3 …

After induction, carboplatin and etoposide stop. Serplulimab alone can continue every three weeks while it is helping and its risks remain acceptable. There is no fixed universal number of maintenance cycles in this NCCP regimen.

Maintenance means ongoing treatment, not treatment without side effects. Your plan can change after discussion.

BASELINE → TREATMENT → FOLLOW-UP

Checks across treatment phases

Relative stages, not equally spaced dates. No patient dates are needed. Pancreatic enzyme checks supplement NCCP from the current product information; the team confirms timing and reviews every result.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00908 v1b · official regimen · EMA Hetronifly SmPC · updated8October2026

THE WHOLE TREATMENT JOURNEY

Serplulimab, carboplatin and etoposide: treatment and checks

First-line extensive-stage small-cell lung cancer

Induction lasts four planned 21-day cycles unless treatment needs to stop earlier. On Day 1 you receive serplulimab, carboplatin and etoposide by drip. With the standard IV schedule, etoposide is repeated on Days 2 and 3. After induction, carboplatin and etoposide stop. Serplulimab alone can continue every three weeks while it is helping and its risks remain acceptable. There is no fixed universal number of maintenance cycles in this NCCP regimen. Your team may instead prescribe etoposide by drip on Day 1 and capsules twice daily on Days 2 and 3. This is a separate alternative, not extra treatment. Oral and IV doses are not interchangeable milligram for milligram. Confirm which plan you have; take the capsules on an empty stomach as prescribed.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Induction Day 1	Induction Days 2–3	Maintenance	After treatment
Treatment NCCP 00908	Confirm suitability and chosen etoposide route	Serplulimab → carboplatin → etoposide	Etoposide IV each day or prescribed oral BID alternative	Serplulimab alone every21days after four induction cycles	Follow-up or next treatment per team
Blood counts, organs and hormones NCCP 00908 · SERPLULIMAB-SMPC	full blood count (FBC), kidney/liver, glucose, thyroid and pancreatic enzymes; baseline suitability	Review symptoms and cycle bloods before dosing	Report symptoms between visits; extra tests if needed	Cycle bloods; thyroid every 3–6 weeks; pancreatic enzymes periodically/ as needed	Follow unresolved changes and the team's plan
Kidneys, hearing and other checks NCCP 00908 · SERPLULIMAB-SMPC	Measured GFR preferred; audiology if indicated	Validate renal dose and recent creatinine	Report low urine, hearing changes or weakness	electrocardiogram (ECG), cortisol, lipids and audiology as indicated	Persistent or new problems still need review
Support and route NCCP 00908 · SERPLULIMAB-SMPC	Confirm IV or oral etoposide route; written anti-sickness plan	Carboplatin-specific high-risk anti-sickness plan	Etoposide on both days; reconcile support through two days afterwards	No routine induction chemotherapy antiemetics	Keep the current medicine and contact plan

Care / check	Before starting	Induction Day 1	Induction Days 2–3	Maintenance	After treatment
Immune effects and cancer review NCCP 00908 · SERPLULIMAB-SMPC	Record symptoms; explain immunotherapy alert card	Report infusion symptoms immediately	Immune symptoms can start any day	Continue symptom vigilance; scans per consultant plan	Delayed immune effects remain possible

What each check is for

1 Baseline and route

Before the first cycle

Confirm suitability and correct schedule

Ask / act: Document IV versus oral etoposide, GFR method, medicines and urgent contacts.

2 Induction to maintenance

After four planned induction cycles

Chemotherapy and routine support stop

Ask / act: Confirm maintenance plan and continue cycle bloods and immune checks.

3 Treatment pause or completion

For toxicity, progression or agreed change

Delayed effects and ongoing care

Ask / act: Give a follow-up, hormone-replacement if needed and urgent-contact plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Appointments

Serplulimab is given first over about 60 minutes at the first visit. If tolerated, later doses may take about 30 minutes. Carboplatin takes about 30 minutes and IV etoposide about 60 minutes. Assessment, preparation and observation add time. Days 2–3 need further etoposide visits unless the separate oral alternative has been prescribed.

STEP 2

Route and phases

Induction lasts four planned 21-day cycles unless treatment needs to stop earlier. On Day 1 you receive serplulimab, carboplatin and etoposide by drip. With the standard IV schedule, etoposide is repeated on Days 2 and 3. Your team may instead prescribe etoposide by drip on Day 1 and capsules twice daily on Days 2 and 3. This is a separate alternative, not extra treatment. Oral and IV doses are not interchangeable milligram for milligram. Confirm which plan you have; take the capsules on an empty stomach as prescribed.

STEP 3

Maintenance visits

STEP 4

Medicines and urgent advice

Keep the written anti-sickness schedule, capsule instructions if relevant, thermometer, immunotherapy alert card and oncology numbers available. Ask early about transport or help at home.

Before the first infusion

Your hospital’s instructions take priority.

Tell the team your starting point

Describe usual breathing, bowel pattern, energy, skin and nerve symptoms. Mention autoimmune illness, previous lung inflammation, infection and all medicines including steroids and supplements. Changes are easier to recognise when the team knows what is normal for you.

Prepare the practical details

Keep the oncology and out-of-hours numbers, a thermometer and the immunotherapy alert card available. Check blood-test appointments, transport and who can help at home. Ask for written medicine directions and accessible information.

What happens on the day?

Serplulimab is given first over about 60 minutes at the first visit. If tolerated, later doses may take about 30 minutes. Carboplatin takes about 30 minutes and IV etoposide about 60 minutes. Assessment, preparation and observation add time. Days 2–3 need further etoposide visits unless the separate oral alternative has been prescribed. Tell the nurse immediately about flushing, itching, chest pain, dizziness, breathing changes or pain around the drip.

What tests and scans check

A normal earlier test does not rule out a new problem.

Blood counts

Neutrophils help fight infection, platelets help stop bleeding, and red cells carry oxygen. Counts can fall between visits. You cannot know a count from the day of the cycle or from feeling well.

Kidneys, liver and hormones

Kidney function helps set carboplatin dosing and may affect etoposide dosing. Liver tests and albumin also matter for etoposide. Serplulimab can inflame organs and hormone glands, so blood glucose and thyroid checks continue during maintenance. Report new symptoms even if an earlier test was normal.

Scans and treatment review

The team sets the scan schedule and reviews scans with symptoms and examination. Stable cancer can be a worthwhile result. Report new symptoms before the next scan; do not assume worsening is evidence that immune treatment is working.

Delays and changed doses

Chemotherapy may be delayed or reduced for toxicity. Serplulimab is held or stopped for relevant toxicity rather than dose-reduced. Ask which medicine is changing and why. Omitted checks are not the same as normal results.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment	 **Days 1–3** Induction Days 1–3 · maintenance Day 1 only
Nausea / appetite	 **Days 1–5** Early days · may continue longer
Blood counts / infection	 **Days 1–21** Counts may fall between visits · timing varies
Energy / mouth / bowels	 **Days 1–21** Symptoms vary across the cycle
Immune effects	 **Days 1–21** Any day · also after treatment ends

Three treatment days during induction: Induction lasts four planned 21-day cycles unless treatment needs to stop earlier. On Day 1 you receive serplulimab, carboplatin and etoposide by drip. With the standard IV schedule, etoposide is repeated on Days 2 and 3.

Maintenance and later follow-up: After induction, carboplatin and etoposide stop. Serplulimab alone can continue every three weeks while it is helping and its risks remain acceptable. There is no fixed universal number of maintenance cycles in this NCCP regimen. Immune effects can begin after treatment ends; keep the alert card and urgent contact instructions.

The oral alternative needs its own prescription: Your team may instead prescribe etoposide by drip on Day 1 and capsules twice daily on Days 2 and 3. This is a separate alternative, not extra treatment. Oral and IV doses are not interchangeable milligram for milligram. Confirm which plan you have; take the capsules on an empty stomach as prescribed.

Why these bands stay broad: The main bands illustrate a 21-day induction cycle with treatment Days 1–3. Maintenance uses serplulimab on Day 1 only. The reference picture does not predict symptom timing or identify safe days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

There are no guaranteed "safe days". Use the oncology emergency instructions for fever, shivers or suddenly feeling unwell, even with a normal temperature. New cough/breathlessness, severe diarrhoea, reduced urine, confusion or severe rash need prompt advice. Severe breathing difficulty, chest pain, collapse or major bleeding need emergency help. Immune symptoms may start after treatment stops.

Between-treatment notes

Day 1: Day 1: three medicines during induction

Serplulimab is given first over about 60 minutes at the first visit. If tolerated, later doses may take about 30 minutes. Carboplatin takes about 30 minutes and IV etoposide about 60 minutes. Assessment, preparation and observation add time. Days 2–3 need further etoposide visits unless the separate oral alternative has been prescribed. Before leaving, confirm Days 2–3 etoposide arrangements, your written support prescription and urgent contact number.

Day 2–5: Etoposide Days 2–3 and early recovery

Your team may instead prescribe etoposide by drip on Day 1 and capsules twice daily on Days 2 and 3. This is a separate alternative, not extra treatment. Oral and IV doses are not interchangeable milligram for milligram. Confirm which plan you have; take the capsules on an empty stomach as prescribed. Anti-sickness support must cover the actual treatment days and afterwards. Take only the written plan; ask before adding duplicate medicines. Contact the team for persistent vomiting, inability to drink, new diarrhoea or a problem taking prescribed capsules.

Day 6–14: Keep checking how you feel

Blood counts may be reduced between treatments. You cannot tell your count from the date or from how well you feel. Fatigue or mouth symptoms may affect eating and daily activity. Use the urgent advice below for fever, shivering or suddenly feeling unwell, including with a normal temperature.

Day 15–21: Prepare for your next review

Some symptoms may settle; others can continue or appear for the first time. Improvement does not prove your blood counts have recovered. The next treatment depends on your review and tests. Bring your symptom record and questions. Confirm the next appointment: the next Day 1 is usually 21 days after this cycle's Day 1, if treatment proceeds.

Support around the medicines

1

Induction: plan each treatment day

Day 1 carboplatin has a stronger anti-sickness plan. Days 2–3 etoposide also need planned cover, with review for two days after the final chemotherapy dose. Ask for one reconciled schedule; do not add another steroid or anti-sickness course yourself.

2

Maintenance: a different prescription

Serplulimab alone usually does not need the routine chemotherapy anti-sickness course. Continue the alert card and immune symptom checks; new nausea or vomiting may need investigation, not just extra anti-sickness tablets.

Etoposide: confirm the route

There are two NCCP schedules. Your prescription chooses one.

Standard IV schedule

Etoposide is given by drip on each of Days 1, 2 and 3 during induction. Serplulimab and carboplatin are given on Day 1. The next cycle is planned 21 days after Day 1, if your team confirms it.

Separate IV and capsule alternative

Your team may instead prescribe etoposide by drip on Day 1 and capsules twice daily on Days 2 and 3. This is a separate alternative, not extra treatment. Oral and IV doses are not interchangeable milligram for milligram. Confirm which plan you have; take the capsules on an empty stomach as prescribed. Capsules are supplied in different strengths; your label specifies the number to take. Do not open, crush or share capsules. Ask the pharmacist about handling, storage and returning unused doses.

Side effects and what to do

Selected possibilities are shown; this is not a prediction or a complete medicine leaflet.

Nausea, vomiting and appetite

Take prescribed anti-sickness medicines as directed. Small meals and regular sips may help. Report poor control early; persistent vomiting, inability to keep fluids down or much less urine needs prompt advice.

Fatigue and weakness

Balance rest with manageable activity. Sudden severe weakness, dizziness, breathlessness or inability to manage basic tasks needs assessment: infection, anaemia or hormone problems can resemble ordinary tiredness.

Mouth and bowels

Use gentle mouth care and report painful ulcers or difficulty drinking. Report new diarrhoea, abdominal pain or blood in stools promptly because immune bowel inflammation is possible. Ask about constipation from anti-sickness drugs or pain medicines; severe pain, swelling or vomiting needs assessment.

Hair, skin, hearing and nerves

Chemotherapy can cause hair loss. Report new hearing changes, numbness or weakness, and tell the team if these affect balance or daily tasks. Serplulimab can cause immune skin reactions: blistering, peeling or sores around the eyes or mouth need urgent advice.

Bruising and bleeding

Report unusual bruising, nosebleeds or bleeding gums. Heavy bleeding, black stools with weakness or severe breathlessness needs urgent assessment. Do not change prescribed blood thinners without advice.

Serplulimab: symptoms that deserve special attention

Immune treatment can cause inflammation in healthy organs, during treatment or after it ends. Keep your immunotherapy alert card and tell other healthcare professionals you have received serplulimab.

Lungs, bowel, liver and kidneys

New cough or breathlessness, diarrhoea, yellow skin or eyes, dark urine, reduced urine or swelling can signal inflammation. These symptoms also have other causes, including infection and cancer. Contact the team promptly for assessment rather than trying to identify the organ or diagnosis yourself.

Hormones, heart and nervous system

Report unusual weakness, persistent headache, dizziness, marked thirst, frequent urination, palpitations, muscle weakness or changes in vision. Severe chest pain, collapse, confusion, seizures or major breathing difficulty require emergency care. Some hormone effects need ongoing replacement treatment even after immunotherapy stops.

Why early reporting matters

The team may pause treatment and investigate. Some reactions need steroids or other immune-suppressing treatment; others need hormone replacement. These are different from the short planned steroid course around chemotherapy. Do not start leftover steroids, change their dose or stop a prescribed course yourself. Carry the treatment name when attending emergency care.

Muscle weakness and vision changes

New muscle weakness, tiring much more easily, drooping eyelids, swallowing difficulty or new visual symptoms need urgent advice. Difficulty breathing or swallowing needs emergency care. Serplulimab can cause serious nerve-muscle or eye inflammation; do not wait for the next appointment.

Living with treatment

The aim is a plan that fits your life as well as your treatment. Bring practical concerns to the team early.

Food, fluids and weight

Aim for enough food and fluid to maintain strength, using foods you tolerate. Tell the team about weight loss, swallowing problems or difficulty shopping or cooking; ask for dietitian support. A fixed fluid target is not right for everyone, particularly with heart or kidney disease. “Anti-cancer” diets, fasting and high-dose supplements are not substitutes for treatment and can create nutritional or interaction problems.

Movement, work and rest

Try activity that feels manageable and increase or reduce it according to symptoms. Short walks or simple daily tasks may be more realistic than your usual exercise. Discuss work adjustments and travel plans. Do not drive if dizzy, unusually tired or affected by sedating medicines. Sudden deterioration should prompt assessment, not an attempt to push through exercise.

Infection precautions without isolation

Wash hands, practise food hygiene and avoid close contact with people who are unwell. Ask about vaccinations and their timing; some vaccines are unsuitable during treatment. You can usually still spend time with family. Keep your urgent contact plan available: precautions reduce risk but do not make fever or sudden illness safe to watch at home.

Intimacy, fertility and pregnancy

Discuss fertility preservation before starting if relevant. Do not become pregnant or breastfeed during this treatment. NCCP specifies effective contraception during serplulimab and for at least six months after the last dose for women of childbearing potential; the team must reconcile all medicines into one written contraception and breastfeeding plan. Report a possible pregnancy promptly.

Emotional support and choices

It is reasonable to feel worried, angry, uncertain or overloaded. Ask for psycho-oncology, counselling, social work or family support. You can request information in smaller pieces, an interpreter or a written summary. Tell the team what you most want to preserve in daily life. You can revisit treatment decisions and ask for more time or another explanation.

A short treatment dictionary

Terms you may hear in clinic.

Cycle · induction · maintenance

A cycle is a planned treatment-and-interval period. Induction is the initial combination phase. Maintenance is ongoing treatment after that phase; it does not mean side-effect-free treatment.

Response · stable disease · progression

Response means the cancer has reduced by the assessment used. Stable disease means it has not changed enough to meet response or progression criteria. Progression means growth or spread meeting the assessment criteria. Your team interprets these alongside how you are feeling.

Neutropenia · anaemia · thrombocytopenia

Neutropenia means low infection-fighting neutrophils. Anaemia means low haemoglobin. Thrombocytopenia means low platelets. These are different results with different implications; one normal number does not establish that all treatment risks are low.

Immune-related · colitis · pneumonitis

Immune-related effects come from immune activity against healthy tissue. Colitis means inflammation of the colon; pneumonitis means inflammation in the lungs. A symptom alone cannot establish either diagnosis.

Questions worth asking

How will we know if it is helping?

Ask your oncologist what the treatment is aiming to achieve for you, when scans are planned and what would lead to continuing, changing or stopping it. How you feel and the amount of side effects do not reliably show how well treatment is working.

Why might a cycle be delayed?

A delay can give time to recover or investigate a new symptom. It is a safety decision, not a personal failure. Ask which medicines are affected, what needs to improve and when you will be reviewed.

Will I get every side effect?

No. The body map shows selected possibilities, not a prediction. Ask about common effects, serious less common effects, and any long-term risks that matter particularly to you. The same symptom can have several causes.

What should I organise before starting?

Confirm blood-test and Day Ward arrangements, transport home, your medicines schedule and a thermometer. Keep the oncology contact instructions and immunotherapy alert card available. Ask for an interpreter or accessible information if useful.

What else belongs in the consent conversation?

Your priorities, expected benefit, alternatives including supportive care, fertility, contraception, pregnancy or breastfeeding, work, driving and the possibility of lasting side effects. You can ask more questions or reconsider treatment at any stage.

Do more side effects mean the treatment is working better?

No. Side effects are not a reliable personal measure of cancer response. Report them so they can be treated; do not tolerate serious symptoms in the hope that they are a sign of benefit.

What should I say when calling about a symptom?

Give the current medicines or say you are in maintenance, the last treatment date, what changed and when, your temperature and whether you can eat, drink and pass urine. Do not delay calling to collect every detail.

Which checks continue afterwards?

Immune effects can arise after the final serplulimab dose. Keep the alert card and urgent contact plan. Follow-up of blood counts, organ changes and scans follows the team's plan; no universal scan interval is imposed by this regimen.

Will I come to hospital on all three treatment days?

Usually with the standard IV induction plan, yes. Some people receive a separate capsule prescription for Days 2 and 3. Confirm the route and exact schedule before arranging transport; never substitute capsules for a drip yourself.

What if I miss or vomit an etoposide capsule dose?

Contact the oncology team or pharmacist for instructions. Do not double the next dose or repeat a vomited dose without advice. Store and handle capsules exactly as the pharmacy advises.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Serplulimab

- Immune inflammation of lungs, bowel, liver, kidneys, skin and hormone glands
- Immune effects can also involve the heart, pancreas, nervous system or blood cells
- Severe infusion reactions
- Some hormone effects need lasting replacement treatment; immune effects can begin after the final dose

Carboplatin

- Low blood counts, infection or bleeding
- Allergic reactions
- Kidney, hearing and nerve effects

Etoposide

- Low blood counts, infection and bleeding
- Hair loss and mouth or bowel symptoms
- Infusion reactions and low blood pressure
- A delayed risk of treatment-related leukaemia

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction: carboplatin Day 1 and etoposide Days 1–3

Active medicines: serplulimab + carboplatin + etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before treatment Day 1, then daily Days 2–3
Ondansetron	16 mg orally	Day 1 Individualise for QT risk and patient factors.
Dexamethasone antiemetic reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	NCCP NCIS V8 high-risk dedicated carboplatin AUC≥4 row One reconciled prescription for the multi-day course. Avoid duplicating corticosteroids when the day-specific etoposide support is reviewed.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS table does not specify an end day here; prescribe duration and suitability. Not a routine olanzapine-containing row.
Days 2–3 etoposide and Days 4–5 review	No automatically added duplicate course; confirm individual day-specific prescription	Before each etoposide day and for two days after the final chemotherapy day NCCP NCISV8 multi-day rule applies. Etoposide is low risk; the generic low-risk reference is ondansetron8mg PO daily with subsequent metoclopramide10mg PO TDS as needed, but it must be reconciled with the existing carboplatin course, QT risk and local policy rather than simply stacked. Day5 coverage must be explicitly reviewed.

Additional phase-specific support

- Use the dedicated carboplatin AUC≥4 high-risk reference (no routine olanzapine): aprepitant125mg Day1 then80mg Days2–3, ondansetron16mg Day1, dexamethasone12mg Day1 then8mg Days2–4; subsequent metoclopramide10mg TDS with duration/suitability prescribed.
- Etoposide is given on Days1–3. Reconcile support for each active treatment day and for two days afterwards; a fixed Day1 script alone is not a complete multi-day prescription.
- No routine infusion premedication is usually required unless there has been a previous hypersensitivity reaction. Have anaphylaxis facilities available.
- If oral etoposide is chosen, confirm capsule strengths, empty-stomach dosing, adherence and handling; no replacement dose after vomiting/missed dose without advice.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Maintenance: serplulimab alone

Active medicines: serplulimab

Emetogenic risk: minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Serplulimab alone has minimal emetogenic potential: no routine continuation of the induction chemotherapy antiemetic course.
- Provide symptom-directed rescue if appropriate; investigate new nausea/vomiting for endocrine, immune and other causes.
- Continue cycle bloods, thyroid tests every3–6weeks and immune symptom vigilance; keep the alert card.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

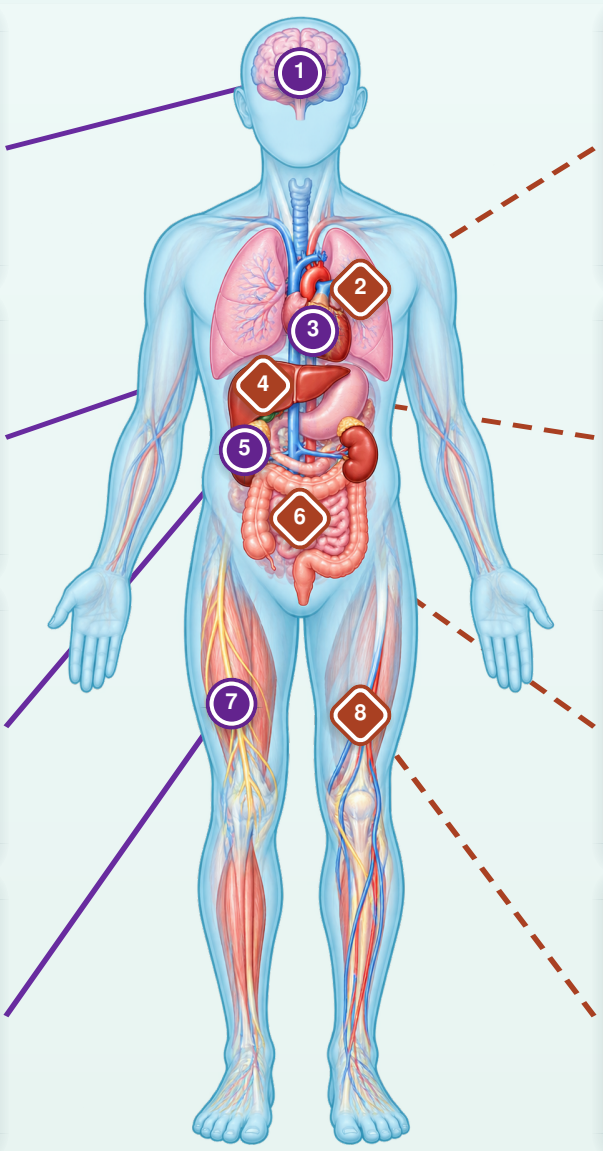
Serplulimab, carboplatin and etoposide

Induction lasts four planned 21-day cycles unless treatment needs to stop earlier. On Day 1 you receive serplulimab, carboplatin and etoposide by drip. With the standard intravenous (IV) schedule, etoposide is repeated on Days 2 and 3. After induction, carboplatin and etoposide stop. Serplulimab alone can continue every three weeks while it is helping and its risks remain acceptable. There is no fixed universal number of maintenance cycles in this NCCP regimen. Your team may instead prescribe etoposide by drip on Day 1 and capsules twice daily on Days 2 and 3. This is a separate alternative, not extra treatment. Oral and IV doses are not interchangeable milligram for milligram. Confirm which plan you have; take the capsules on an empty stomach as prescribed.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Hormones / nervous system

Serplulimab

Severe weakness, headache, dizziness, thirst, confusion or seizures need prompt or emergency assessment.

2 Lungs

Serplulimab

New cough or breathlessness: contact the team; severe breathing difficulty needs emergency help.

3 Heart

Serplulimab

Chest pain, racing heart or collapse need urgent assessment.

4 Liver

Serplulimab / etoposide

Yellow skin or eyes, dark urine or new severe nausea need advice; tests can detect changes before symptoms.

5 Kidneys

Carboplatin / serplulimab

Much less urine, swelling or persistent vomiting need advice; kidney function affects doses.

6 Bowel / pancreas / mouth

Serplulimab / chemotherapy

New diarrhoea, severe abdominal pain, blood in stool or inability to drink need prompt assessment.

7 Skin / nerves

Serplulimab / chemotherapy

Blisters, peeling, mouth or eye sores, new weakness or numbness need advice.

8 Blood / infection

Carboplatin / etoposide; rarely immune effects

Fever, shivering or sudden illness: call oncology immediately. Report unusual bleeding or bruising.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00908 v1b · official regimen**
   https://healthservice.hse.ie/documents/6856/908_NCCP_systemic anticancer therapy (SACT)_Regimen_Serplulimab.pdf
2. **EMA Hetronifly SmPC · updated8October2026**
   https://www.ema.europa.eu/en/documents/product-information/hetronifly-epar-product-information_en.pdf
3. **NCCP NCIS Medical Oncology antiemetic medicines V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
4. **EMA Hetronifly overview and current product-information link**
   https://www.ema.europa.eu/en/medicines/human/EPAR/hetronifly
5. **ASTRUM-005 long-term secondary analysis · JAMA Oncology 2026**
   https://pubmed.ncbi.nlm.nih.gov/42240984/
6. **ASTRUM-005 original randomised trial · JAMA 2022**
   https://jamanetwork.com/journals/jama/fullarticle/2796691
7. **October 2026 correction: omitted eTable 6 restored in Supplement 3**
   https://jamanetwork.com/journals/jamaoncology/fullarticle/2854861
8. **HSE chemotherapy side effects and urgent contact advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **HPRA Vepesid oral product information**
   https://assets.hpra.ie/products/Human/30580/Licence_PA2239-006-001_27022024163103.pdf
10. **HPRA etoposide product information cited by NCCP00908**
    https://assets.hpra.ie/products/Human/27660/Licence_PA2315-201-001_13032024133422.pdf